

Patient 21**Report A**

6564449 13/07/2016 CT Acute Stroke

Indication: Right MCA infarct ?stenosis

CT HEAD

Comparison has been made with CT head 13/07/2016.

Haemorrhage: No

Intravascular thrombus: No Acute infarct: Yes, right temporoparietal wedge-shaped area of low attenuation in keeping with subacute infarct. There is mild surrounding mass effect, no complication of parenchymal haemorrhage. Other findings and comments: There is a right transparietal VP shunt with tip crossing the midline and located in the mesial aspect of the left lateral ventricle body. There is circumferential non-stenosing cavernous/supraclinoid ICA atheroma.

CT ANGIOGRAM

Window settings: (intraluminal HU=220; W=440; L=165)

(ICA Stenoses NASCET method, measurements on axial 1mm sections) Aortic arch and subclavian arteries: Mild mixed arch atheroma, Left-sided aortic arch with conventional anatomy and subclavian Origins of the vertebral arteries.

RCCA/ICA: Minor carotid bifurcation mixed atheroma, no significant stenosis.

LCCA/ICA: There is mild carotid bifurcation mixed atheroma extending into the proximal internal carotid arteries, no significant stenosis.

RVA: Normal

LVA: Hypoplastic Intracranial Arteries: The proximal branches at the circle of Willis are patent with no focal stenosis, occlusion or acute dissection. Bilateral near foetal PCA is.

Other findings and comments:

There is bilateral lower pole thyroid nodules which can be further characterised with ultrasound if clinically indicated. Visualised lung apices are clear.

Conclusion: Subacute right MCA territory infarct with mild surrounding mass effect, no definite complication parenchymal haemorrhage. The cervical carotid/vertebral arteries and Circle of Willis are patent with no focal stenosis, occlusion or acute dissection.

Report B

6564.866 14/07/2016 MR Head

Clinical indications for MRI - Collapsed on Friday. Unsteady since then. P R MCA infarct? right transparietal VP shunt well positioned

Findings:

Comparison to previous CT of 13/316.

Artefact on the right transfrontal VP shunt limits image interpretation.

Although partly obscured by adjacent artefact an acute right parietotemporal and insular infarct is confirmed on MRI. Further small acute cortical infarcts are seen in the left inferior occipital lobe. A focus of high DWI trace signal without

ADC reduction is seen in the left frontal subcortical white matter, suggestive of a small subacute infarct at this site, No abnormal parenchymal susceptibility is identified to suggest haemorrhage. There is an old right thalamic lacune with tiny bilateral cerebellar hemispheric infarcts. This is seen on a background of moderate presumed small vessel disease related white matter signal abnormality.

The time-of-flight MRA shows focal poor flow related enhancement in a posterior right M2MCA branch, suggestive of intravascular thrombus. Furthermore the proximal aspect of the superior division of the left P3 PCA shows poor flow related enhancement which could relate to intravascular clot, this was more evident on the CTA. There is a probable longstanding occlusion/high-grade stenosis of the post PICA left V4 vertebral artery.

Moderate cervical spondylotic changes are noted with degenerative anterolisthesis of C3 on C4.

Consultant addendum

I agree with the above report. The left V4 vertebral artery narrowing may be constitutional. Note is also made of a stenosis at the left M1M2 junction.